

equicenta[®] CTM

Cryopreserved Equine Umbilical Cord Tissue Allograft

For intra-articular, intralesional, and perilesional use in joints, tendons, and ligaments — characterized in BMC Medicine (2025) and evaluated in a pragmatic, prospective, controlled clinical trial peer-reviewed in Frontiers in Veterinary Science (2026).



FOR VETERINARY PROFESSIONAL USE

138 HORSES • 195 CONDITIONS • 8 PRACTICES • 26 WEEKS



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Largest Controlled Trial of Its Kind in Equine Regenerative Medicine

A published controlled clinical trial in performance horses. In this trial, a single dose of equicenta[®] CTM showed 2.72:1 odds of returning to or exceeding prior performance versus clinician-selected standard of care ($p < 0.006$).



One Dose. A Controlled Trial. A Clear Performance result.

Published Clinical Trial · Bertone et al., Front. Vet. Sci. 2026
One dose. A controlled trial. A clear performance result. **equicenta®**
CTM is a cryopreserved equine umbilical cord tissue allograft
developed for use in equine joints, tendons, and ligaments —
administered as a single, ready-to-use intra-articular, intralesional, or
perilesional injection.



Evidence At A Glance

A first-in-class, cryopreserved equine umbilical cord tissue allograft for joints, tendons and ligaments. In the first controlled trial of its kind in horses, a single ready-to-use injection outperformed standard of care biologics across, pain, swelling, lameness and return to performance - with a favorable safety profile.

2.72:1

Odds of returning to work or exceeding prior performance vs clinician-selected standard of care. Pre-specified 26-week performance endpoint ($p<0.006$).

d=2.23

COHEN'S D — LARGE EFFECT
Preserved in ITT & per-protocol

GREATER CUMULATIVE CLINICAL IMPROVEMENT IN PAIN, SWELLING & LAMENESS

Through Week 12

Clinical outcomes $p<0.003$ through 12 weeks. Clinical outcomes (Week 12) are assessed separately from the 26-week performance endpoint.

138 Horses Enrolled	195 Conditions Treated	8 U.S. Practices 11 Veterinarians	26 Weeks Follow-up
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Pragmatic, prospective, multicenter, controlled trial versus clinician-selected standard of care.



What equicenta® CTM is & What Sets It Apart

equicenta® CTM

equicenta® CTM is a cryopreserved equine umbilical-cord-tissue allograft developed for **intra-articular, intralesional, and perilesional** administration in **equine joints, tendons, and ligaments**. Supplied as a single, **ready-to-use** dose, it eliminates the need for an autologous blood draw, chairside processing, or corticosteroid administration.

Unlike an acellular fluid, such as plasma, that disperses following injection, **equicenta® CTM** retains a native **fetal extracellular-matrix scaffold** at the treatment site. This scaffold contains a characterized multimodal **structural proteins, growth factors, cytokines, and extracellular vesicles**. The retained matrix is designed to provide structural support and progressively release its biological components over time, supporting a local environment associated with the body's natural tissue-repair processes.

Fetal

Age 0 Tissue Source

~134 nm

Mean EV Size

ECM

Native Retained Scaffold

Single

Ready-to-Use Dose

Setting Standards

Defining Attribute Structural ECM scaffold

A native extracellular-matrix framework that persists at the treatment site — a lattice for host-cell migration and repair — rather than a fluid that disperses.

2,645
PROTEINS
IDENTIFIED

4.7%
WITHIN-BATCH CV
ON HIGH-ABUNDANT
PROTEINS

Protein reservoir effect

As the scaffold biodegrades, key factors such as **IGF-1, A2M, and TIMP-2** are released over time; characterizing sustained release in addition to a single burst at injection.

Anti-inflammatory profile

Cytokine profiling showed decreased inflammatory indices alongside **increased anabolic and anti-catabolic factors**.

Timed release

Biodegradation can release the protein over days to weeks in vitro at body temperature.

Extracellular vesicles

population characterized by particle size and EV-associated markers, including CD9, CD81, CD63, and ALIX.

Cryopreserved & Ready-To-Use

Why This Matters

Unlike cell-free fluids without a retained structural matrix, **equicenta® CTM provides a native extracellular-matrix scaffold** together with a characterized multimodal structural proteins, growth factors, cytokines, and extracellular vesicles. The retained matrix is **designed to support the local tissue-repair** environment while the product's composition, handling, and single-dose administration provide a differentiated regenerative-medicine option for veterinarians.



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What Is Actually In The Vial: A Single, Consistent Biological Response

equicenta[®] CTM

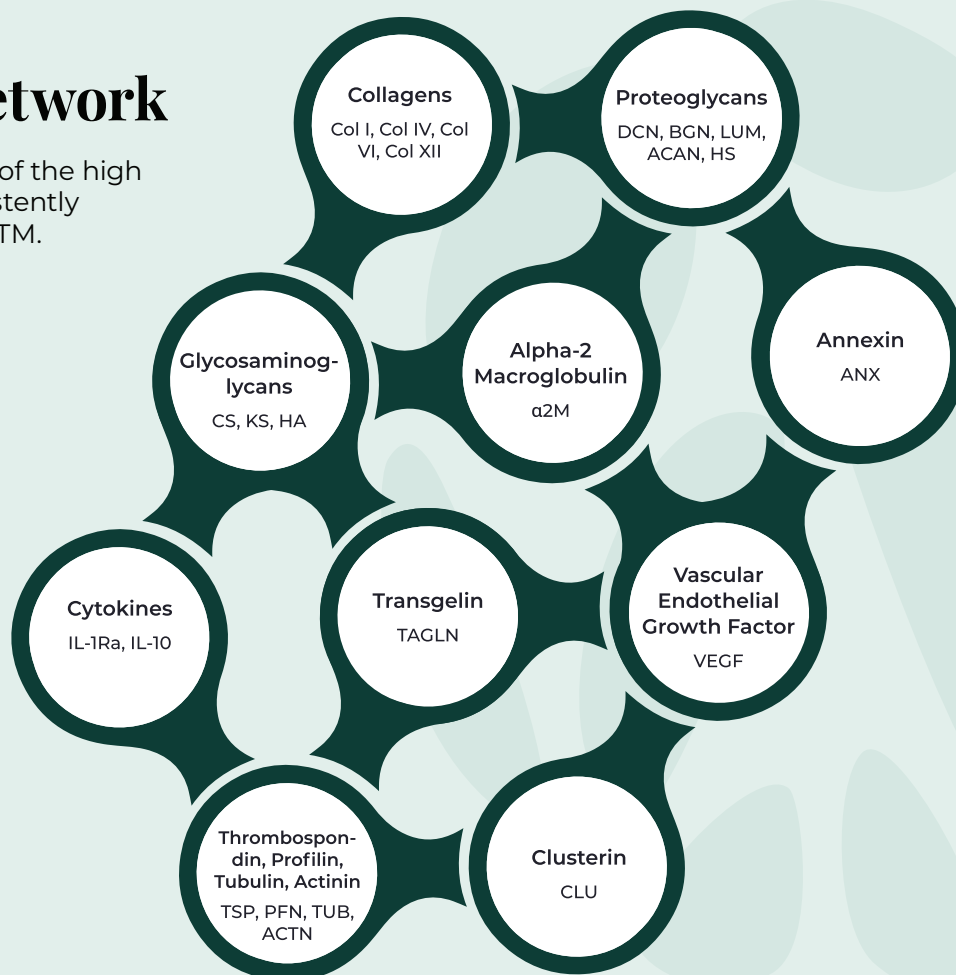
Proteomic Network

A representative selection of the high abundance proteins consistently identified in equicenta[®] CTM.



Learn More

Discover the Biologic Breakdown: how proteins, cytokines, growth factors, and extracellular matrix components come together within equicenta[®] CTM.



Batch-To-Batch Consistency

TESTED ACROSS 11 DONORS

The same high-abundance proteins were reproducibly identified donor-to-donor by total spectral count. A low **4.7% within-batch CV** in high-abundance proteins supports consistent vial-to-vial composition.

- ✓ **80 highly abundant proteins** (TSC ≥ 100) shared across donors
- ✓ Structural, regenerative & regulatory classes all represented
- ✓ A defined biologic — not a crude lysate

Signature Regenerative Proteins

α2-Macroglobulin

Broad protease inhibitor associated with regulation of proteolytic activity

Heparan Sulfate · Aggrecan

Core cartilage & matrix proteoglycans for structural support

IGF-2 · TGFβ1

Growth factors driving cell proliferation & tissue repair

Decorin · Biglycan

Small leucine-rich proteoglycans that organize collagen & modulate healing



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Proteomic Network Representative

High-abundance proteins consistently identified across donors

*A characterized, reproducible biological response across analyzed donors —
not a variable donor-dependent extract.*

Proteins Per Category

Key Functional Categories

Structural Support

Collagen α · Collagens VI/VII/XII · Tubulin · Plectin · Actinin · Tenascin

Glycan / Proteoglycan

Heparan Sulfate · Aggrecan · Decorin · Biglycan · Lumican · Versican · Hyaluronic Acid

Tissue Repair & Regeneration

Annexin A1 · Thrombospondin · Profilin · Calpain · PRELP · Asporin

Anti-inflammatory Regulation

α 2-Macroglobulin · Annexin (Lipocortin) · Galectin · Clusterin · DDAH2

Antioxidant Protection

Superoxide Dismutase · HSP90 · Clusterin · α 2- Macroglobulin

Cellular Homeostasis

HSP90 · Transgelin · Tubulin · Calpain

Energy Generation

Actinin · Tubulin · HSP90

Cell Adhesion

Vinculin · Integrin- α (3 isoforms) · Cadherin · TGFB1 · Thrombospondin

Growth Factors

IGF-2 · TGFB1 · Transgelin · VEGF

2,645

Proteins Identified

11

Donors Analyzed



4.7%

Mean Within-Batch CV*

300+

Abundant Proteins

*On high-abundant proteins



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Natural at the Source

Elevating Equine Care Through Mother Nature's Own Design

Nature's Richest Source

Umbilical cord tissue delivers a naturally concentrated profile of extracellular matrix proteins, growth factors, and cytokines present at birth—shown to be enriched compared to adult tissue sources.

Age-Zero Advantage

Collected at birth, prior to cumulative environmental and mechanical stress—supporting a more consistent, well-preserved biological profile.

Immune-Privileged by Nature

Rich in extracellular matrix components and signaling proteins associated with low immunogenicity, helping maintain a balanced local tissue environment. These naturally occurring properties of perinatal tissue are preserved through controlled cryopreservation.²

Ethically Sourced, No Harm

Recovered following normal, full-term births with no intervention to mare or foal, minimizing collection-related stress and supporting preservation of the native tissue profile under standard veterinary oversight.

From Byproduct to Breakthrough

Transforms normally discarded birth tissue into a clinically relevant biologic, applying established regenerative medicine principles to deliver a purposeful, modern solution.



equicenta® CTM

Sourced from Champions

Coteau Grove Farms was founded in 2008 by Keith and Ginger Myers, driven by their passion for horses and a vision for excellence in thoroughbred breeding. Their journey began with the purchase of eight broodmares at the Keeneland sale in November 2008, laying the foundation for a top-quality breeding program focused on Louisiana-bred racehorses.

Today, Coteau Grove Farms is home to Louisiana champions and graded stakes winners, making a mark both locally and on the national racing stage.

National Recognition for Our Sourcing Partner

2026 TOBA National Breeder of the Year Finalist

Coteau Grove Farms is recognized among the nation's top Thoroughbred breeding operations.



OUR EXCLUSIVE SOURCING PARTNER



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ISO 13485:2016
 Certified
 Medical Devices
 Quality Management



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A Micro-structural Scaffold With a Triple-Action

One implant, three coordinated actions characterized in the response: Builds Tissue, Blocks Breakdown, Calms Inflammation

Anabolic

Growth factors associated with tissue repair and matrix synthesis.

IGF-2 · VEGF · TGF- β · FGF

Anti-Catabolic

Anti-Catabolic Enzyme inhibitors associated with regulation of matrix-degrading enzyme activity.

TIMP-1 · TIMP-2 · A2M

Anti-Inflammatory

Anti-Inflammatory Cytokine profiling showed a regulated balance of inflammatory mediators.

IL-1ra · IL-10

Confirmed

EVs/Exosomes

9.28×10^{11}

EVs / mL (CD9)

~134 nm

Mean Particle Size

15–36%

CD81 + Double Positive

53–83%

CD63/CD9 Co-Capture

Tetraspanins

CD9 / CD81 / CD63 — canonical vesicle surface markers.

ALIX (PDCD6IP)

Cargo marker consistent with true EVs, not cellular debris.

CD90 (MSC)

A CD90+ subset — MSC-associated surface marker.

What This Means

The marker profile supports the presence of extracellular vesicles carrying characterized signaling cargo. Each vial is characterized for a multimodal composition that includes potent growth factors, cytokines, and proteins within a native extracellular-matrix scaffold. Together, these components are intended to support the biological environment associated with the body's own tissue-repair and regenerative processes.

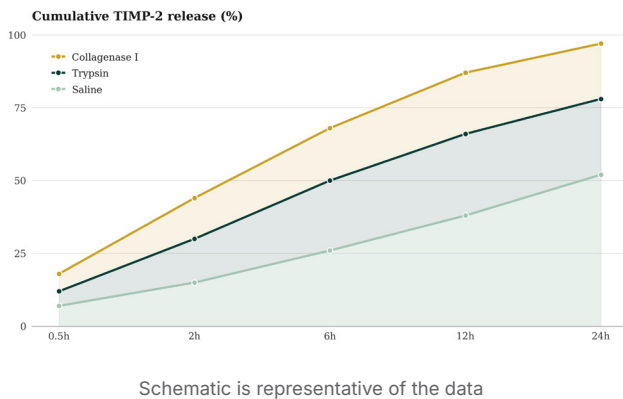


A Timed Release — Not Only a Burst at Injection

In vitro, matrix digestion produced progressive release of characterized factors — supporting a protein-reservoir model rather than a single bolus.

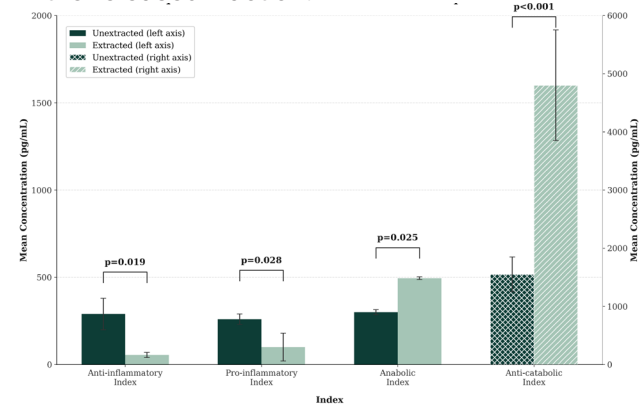
TIMP-2 Release Kinetics

Cumulative TIMP-2 release over time in saline, collagenase I, and trypsin conditions.



Cytokine & Growth-Factor Indices

Relative anti-inflammatory, pro-inflammatory, anabolic, and anti-catabolic indices characterized in the released fraction.



equicenta® CTM

Key Proteins Identified

IGF-1

Insulin-like Growth Factor 1 Increased after extraction. An anabolic growth factor with known roles in cell proliferation and cartilage matrix synthesis.

A2M

Alpha-2 Macroglobulin Increased in the released fraction. A broad protease inhibitor with known roles in regulating proteolytic enzymes (e.g., MMPs, ADAMTS).

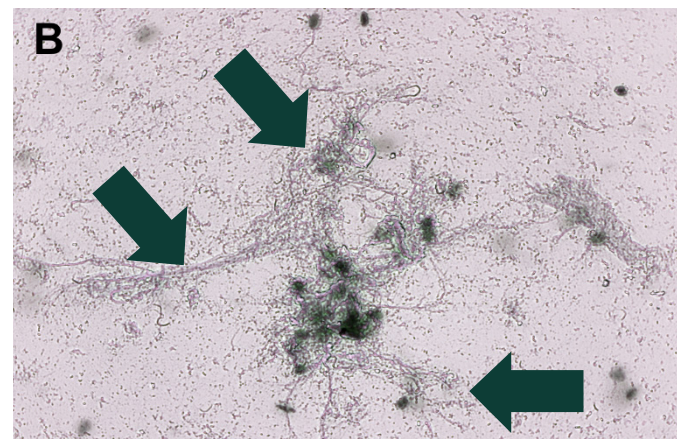
TIMP-2

Tissue Inhibitor of Metalloproteinase 2 Released over time as the matrix degrades. Known to inhibit collagen-degrading metalloproteinases (MMPs).

Take-Home For Veterinarians

In vitro, matrix digestion produced progressive release of characterized factors including IGF-1, A2M, and TIMP-2. These results support a protein-reservoir model and biological plausibility.

Cytological evaluation of pelleted equicenta® CTM fixed with cold methanol:acetone and stained with hematoxylin. Note the presence of filamentous-form matrix (green arrows) observed with pink background staining indicating a proteinaceous extracellular matrix creating a net-like protein scaffold structure. Bar (B)=400 um.



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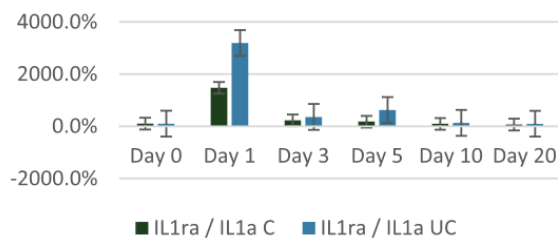
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Inside The Joint: A Brief, Normal Response, Then An Anti-Inflammatory Profile

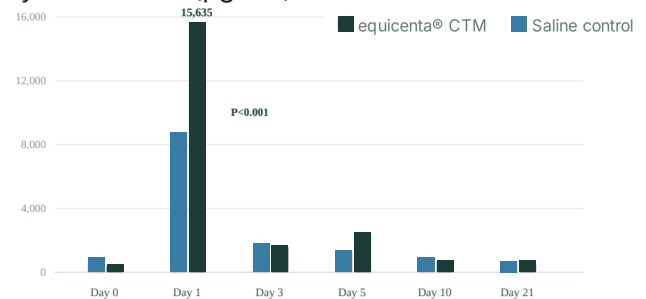
In a two-dose, masked, randomized crossover biocompatibility study in 8 clinically normal horses over 42 days, a single injection triggered a short, self-limiting local response — followed by a sustained anti-inflammatory synovial profile, with no systemic signal.

0Adverse Events
42 Days**400%**IL-1ra/IL-1
Ratio**62%**IL-1ra/IL-1α ratio
Above Saline Control at Day 1**<100**SAA
No Systemic Signal

C - IL1-ra / IL-1a Ratio
Percent Difference from
Baseline Average



IL-1ra — Anti-Inflammatory Surge
Synovial IL-1ra (pg/mL) · Table 5 · P<0.001



Key Findings

The Joint Builds Its Own Anti-Inflammatory Profile

Synovial IL-1ra surged to 15,635 at Day 1 (P<0.001) and remained elevated in 7 of 8 horses the full study, and drove the IL-1ra/IL-1α ratio to 16.4 — an anti-inflammatory, pro-repair signature, not lasting inflammation.

IL-1ra ↑ p<0.001

IL-10 ↑ p<0.01

VEGF ↑ p<0.01

IL-1ra Day 0 → Day 1 after UC: 438 → 15,635 pg/mL

Confirmed In Diseased Joints Too

In a parallel case series of 3 horses with osteoarthritis and desmitis/tendonitis (28 days), **every horse improved ≥1 grade in lameness/pain with 0 adverse events**. In these inflamed joints the IL-1ra/IL-1α ratio moved from a pro-inflammatory 0.3 to >4- fold above control — the same conversion seen in healthy joints. IL-10, HA and VEGF also rose; SAA and imaging stayed stable.

From Biocompatibility to Clinical Evidence

A **characterized product and a favorable biocompatibility profile** established the foundation. The next question was “Would the product work?” Would these properties translate into measurable outcomes versus the non-steroidal biologics already used in practice?

CONTINUE TO
CLINICAL TRIAL →

This controlled biocompatibility study does not establish efficacy. equicenta[®] CTM is investigational; absolute safety has not been established.

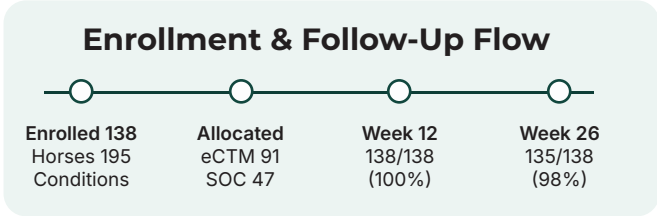


Built on Evidence, Designed to Find Superiority

Product characterization and biocompatibility established the biological rationale. The multicenter controlled clinical study then tested whether that rationale translated into meaningful clinical outcomes — in pain, swelling, lameness, and return to or exceeding prior performance versus clinician-selected standard of care controls.

138 Horses Analysis Cohort	195 Conditions Joint • Ligament • Tendon	8 U.S. Practices 11 Veterinarians	100% Week 12 Complete 138/138	26 Week Follow-Up Performance Window
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Design	Pragmatic, prospective, multicenter , randomized, controlled trial across 8 U.S. specialty equine practices (Oct 2024–Aug 2025), reported under CONSORT with an intention-to-treat analysis and a 26-week per-protocol performance analysis .
Allocation	Randomized within practice by coin-toss plus a block sequence (TTC / TCT / CTT) targeting roughly 2:1; a 1.9:1 eCTM-to-SOC ratio was achieved (91:47). The horse was the unit of assignment.
Comparator	Clinician-selected, non-steroidal standard-of-care biologic (APS, PRP, ACS/IRAP, HA and others) per veterinary judgment — a real-world, head-to-head field comparison , not a saline placebo.
Conditions	All 195 conditions across five diagnostic categories — osteoarthritis/synovitis, tendonitis (SDFT, DDFT), desmitis (suspensory branch, proximal suspensory), OCD, and subchondral bone/cysts — balanced between arms.
Population	No significant between-arm differences in breed, age, sex, weight, discipline or limb (mean 8.7 yr, 490 kg). Thoroughbred 39 · Warmblood 33 · Quarter Horse 33 (of 138).



How Outcomes Were Measured

Veterinarians scored lameness (AAEP 0–4), pain, and swelling at each visit; performance at 26 weeks scored 1–4 (4 = returned to or exceeded previous work). Selected secondary measures (kinematics, imaging) were analyzed blinded to treatment.

CONSECUTIVE ENROLLMENT	STRATIFIED ALLOCATION	BLINDED SECONDARIES	INDEPENDENT BIOSTATISTICS	CONSENSUS SCORER TRAINING	CONSORT REPORTING
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Why This Design Matters

A controlled trial asks a harder question than a case series: how did this product compare with the biologics already used in practice, under the same follow-up rules? **Near-complete follow-up (100% at Week 12; 98% at Week 26)** means the result isn't driven by keeping only the successful cases. Horses were enrolled sequentially, and treatment groups were followed contemporaneously, with allocation that was randomized. A true reflection of how a veterinarian actually decides in practice.



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Greater Clinical Improvement & Greater Return to Performance

A single dose of equicenta[®] CTM vs clinician-selected standard-of-care biologics, peer-reviewed in Frontiers in Veterinary Science, 2026.

2.72:1

Return-To-Performance Odds
p<0.006

d = 2.23

Performance Effect
Cohen's d
Large · 26-Week Endpoint

5.9:1

Fewer Reported Reactions in
eCTM vs SOC
Odds · Central · p<0.002

p<0.003

All Clinical Outcomes
Greater Through Week 12

26-week Performance Effect Size Vs Standard Reference Thresholds

observed d = 2.23 (well beyond "large")

“ *In real-world lame performance horses, a single dose of equicenta[®] CTM improved clinical outcomes and return to performance compared with clinician-selected standard-of-care biologic therapies through 26 weeks* ”

From The Peer-Reviewed Publication

01	CLINICAL	Greater cumulative improvement in pain, swelling, and lameness through Week 12 (all outcomes p<0.003); separation by Week 2 (pain/swelling) and Week 4 (lameness).
02	PERFORMANCE	Large eCTM performance effect at 26 weeks (Cohen's d = 2.23); higher odds of returning to or exceeding prior performance (OR 2.72, p<0.006).
03	SAFETY	Fewer reported injection reactions than SOC (vet review 4.8:1; blinded central review 5.9:1). Per-injection reactions 3.4% eCTM vs 10.5% SOC (central review).
04	CLIENT-REPORTED	Week-26 client-reported scores favored equicenta[®] CTM for body condition, hair coat, overall health, and the quality-of-life index (p<0.03).
05	STRUCTURE	Responding tissue-remodeling score in 8/22 (36%) eCTM vs 0/11 (0%) SOC ligaments on ultrasound (p<0.013) — a supportive, hypothesis-generating signal.



THE PIVOTAL TRIAL · CLINICAL EFFECTIVENESS

Earlier Separation — Sustained Through the Week-12 Window

Cumulative lameness, pain, and swelling versus clinician-selected standard of care.



Baseline
Enroll

Week 2
Pain $p < 0.02$
Swelling $p < 0.02$

Week 4
Lameness
 $p < 0.01$

Week 12
All clinical
 $p < 0.001$

Week 26
Performance
 $p < 0.001$

equicenta® CTM

Better in All Outcomes

>1.6:1

Soundness Odds
Directional

Wk 2

Pain & Swelling
First Separation

Wk 4

Lameness
Significant

3:1

Pain-Free Odds
Week 12

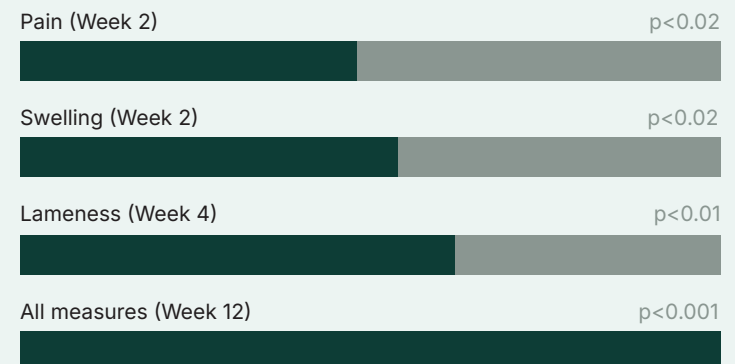
$p < 0.001$

All Clinical Outcomes
Greater Through week 12

Significance by Measure and Timepoint

Cumulative Improvement

Bar length reflects the reported strength of the between-arm difference (manuscript Figure 2 caption).



Clinical Interpretation

equicenta® CTM treated cases showed earlier between-arm separation on scored lameness, pain, and swelling and maintained greater cumulative improvement through the Week-12 window as well a greater performance improvement (dependent on soundness) sustained to 26 weeks showing durability of the positive effect.

At 12 weeks **equicenta® CTM treated horses were three times more likely to be pain free** when compared to Standard of Care.



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Return To Or Exceeding Prior Performance

At the pre-specified 26-week endpoint, **eCTM cases had 2.72:1 odds of returning to or exceeding prior performance** versus clinician-selected standard of care ($p < 0.006$; Cohen's $d = 2.23$).

Return to performance is a pre-specified secondary endpoint, assessed separately at Week 26.

How Much More Likely

79% v 21%

Odds of returning to or exceeding prior performance vs SOC

What Are The Odds

2.72X

Odds of returning to or exceeding prior performance vs SOC ($p < 0.006$)

Effect Size (ITT, 26 Wks)

d = 2.23

Preserved in ITT and per-protocol ($p < 0.001$)

Within-Arm Success Rate

Composition of Score-4 Responders

Share of the **57 horses reaching Score 4 (return to or exceeding previous work)** at 26 weeks.



Clinical Interpretation

Endpoint Definition

Performance score of 4 = return to or exceeding pre-injury level of work, at 26 weeks.

Finding

Composition of responders **among the 57 horses who reached score 4, 79% (45/57) were in the eCTM arm** and 21% (12/57) were in the SOC arm.

equicenta® CTM

One injection.
A clearer path back to work.



Magnitude

Odds of returning to or exceeding prior performance **2.72:1 favoring equicenta® CTM** ($p < 0.006$); large effect ($d = 2.23$).



Fewer Reported Injection Reactions Than Standard of Care

Head-to-head observation versus clinician-selected standard of care in the same trial.

3.4%

eCTM Reactions
5/148 Injections

10.5%

SOC Reactions
8/76 Central

4.8:1

Vet-Reported Reactions
SOC Higher · $p < 0.04$

0/148

Injection-Site Inflammation
eCTM · No Infection

Central Review

Per-Injection Reaction Rate

equicenta® CTM

5/148

3.4%

Vet-Reported Reactions

Standard of Care

8/76

10.5%

Vet-Reported Reactions (Without PAAG reactions included)



Clinical Interpretation

In this trial, clinician-selected standard-of-care biologics were associated with several-fold higher reported injection-reaction odds than equicenta® CTM, and **no serious product-related events were attributed to equicenta® CTM.**



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Client-Reported Outcomes & Performance Convergence with Objective Imaging and Gait Data

Client-reported secondary endpoints at Week 26, converging with Week 12 structural imaging and blinded gait kinematics.

A. Client-Reported Secondary Outcomes (Week 26)

p<0.004

Body Condition
0–10 client scale

p<0.029

Hair Coat
0–10 client scale

p<0.001

Overall Health
0–10 client scale

p<0.03

Quality-of-Life Index
0–70 Summed

B. Structural Healing — Tissue Remodeling

equicenta[®] CTM · Responding TRS

8/22 ligaments

36%

Standard of care · Responding TRS

0/11 ligaments

0%

What the Imaging Showed

Ligaments showed the highest remodeling score with a Cohen's d = 1.25 versus control.

Quantitative Ultrasound (TRS):

B-mode at Weeks 0, 2, 4 & 12; six texture features combined into a composite score.

Rescue Crossover

After Week 12, 19 horses (13%) met the prespecified rescue criterion (no improvement) and opted for rescue injections. **Horses crossed from SOC to eCTM recovered to a level indistinguishable from first-line eCTM (p=0.87) by 26 weeks**, while horses re-treated SOC→SOC continued to decline (p<0.001).

C. Blinded Sleip Smartphone Gait Kinematics

METRIC	TIME	RESULT
Forelimb impact	Week 4	p<0.05
Forelimb push-off	Week 2	p<0.003
Forelimb push-off	Week 12	p<0.01
Combined fore + hind, impact ¹	Week 12	p<0.026

¹ Combined fore + hind data were more variable than forelimb-only data and were flagged in the manuscript as underpowered for further between-group analysis. Analysts blinded to treatment; Weeks 2–12.

Objective Method

Horses receiving eCTM showed steady improvement (lower values) in lameness over 12 weeks that were statistically significant for impact at 4 weeks (p < 0.05) and push-off at 2 (p < 0.003) and 12 (p < 0.01) weeks compared to SOC.

Why It Matters

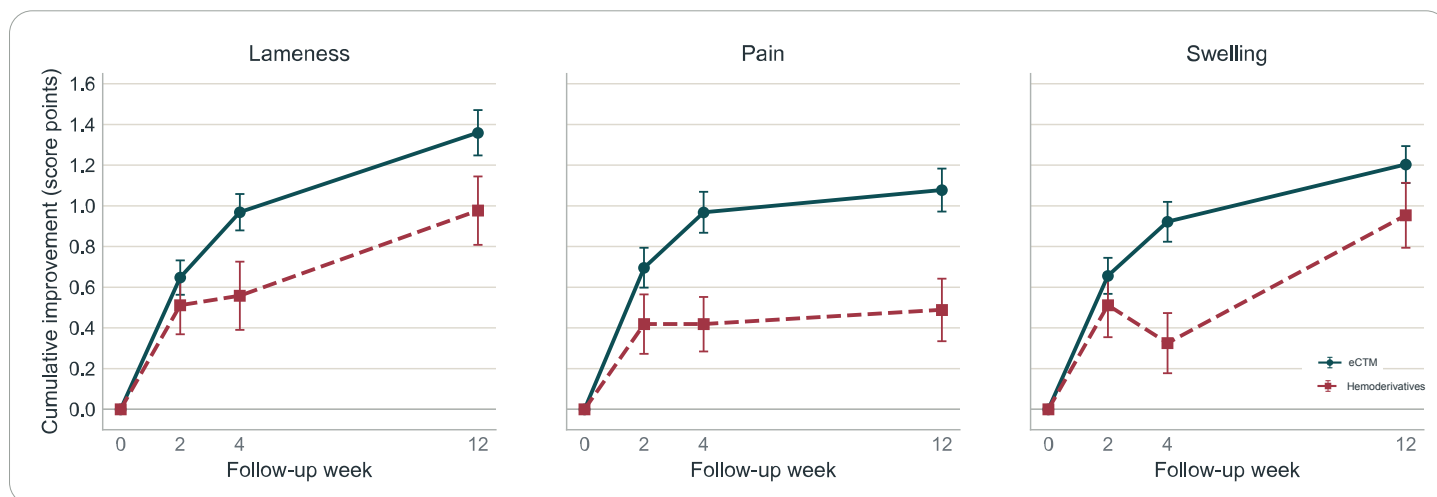
Two blinded and quantitative modalities showed directionally consistent signals; forelimb kinematics was the most stable functional readout.



Clinical Outcomes Compared With Hemoderivative Class of Biologics

How a single dose of eCTM performed against the blood-derived biologics vets use most — across a 12-week clinical window, in the largest treatment subgroup of the standard-of-care arm.

Cumulative Improvement from Baseline - eCTM Vs Hemoderivatives (63.2% of SOC)



Against the hemoderivative subgroup class, a single eCTM injection produced greater cumulative improvement in lameness (Weeks 4 & 12; $p < 0.04$), pain ($p < 0.003$), and swelling ($p < 0.001$) through Week 12.

$p < 0.04$

Lameness
Weeks 4 & 12

$p < 0.003$

Pain
12 Week Trajectory

$p < 0.001$

Swelling
12 Week Trajectory

The Comparator that Matters

Blood-derived orthobiologics — APS, PRP, ACS, and PPP — were the largest single subgroup of the clinician-selected SOC arm. In this predefined subgroup, eCTM showed greater cumulative improvement.

APS Inset: The Product With The Most Prior Evidence

Post hoc analysis - APS was the largest single product class within the hemoderivative subgroup (**$n=22$; 51% of hemoderivatives**) and had prior short-term supportive evidence in equine joint osteoarthritis. In this analysis, eCTM remained significantly associated with greater improvement at multiple time points, primarily in **lameness and pain**.

Single Dose Vs Injection Burden

Each eCTM condition received a single injection, with clinical improvement measured through Week 12. SOC horses received **significantly more re-injections to reach their reported improvement ($p < 0.03$)**. Repeat joint injection carries its own risks and considerations including infection.



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Storage, Handling & Administration

Ready-to-Use administration — no autologous blood draw, centrifugation, or chairside processing or maintenance of centrifuges and bulky kits for a multitude of products.

600+ Doses Administered

Clinical Use to Date

65+ Practices

Have Administered equicenta[®] CTM

~10 Min Thaw

From -80°C , No Processing

1 Single Product

Joint, Tendon & Ligament Use



Handling & Administration

- ✓ Single 1.5 mL dose for intra-articular, intralesional, or perilesional administration
- ✓ Cryopreserved at -80°C ; ~10-minute thaw to administration
- ✓ One product across joint, tendon, and ligament indications, including suspensory desmopathy

A Typical Treatment Step-By-Step

- 1 Remove a vial from -80°C storage and thaw in hand (~10 minutes).
- 2 Resuspend and draw up; no blood draw or centrifugation required.
- 3 Administer at the target joint or lesion under standard aseptic technique.



equicenta[®] CTM



“He completely tore his suspensory branch, and I was told he would never be ridden again. It was devastating. But after the clinical trial with equicenta[®], his recovery was crazy fast—about four months—and I was back on him. It honestly felt like a miracle.”

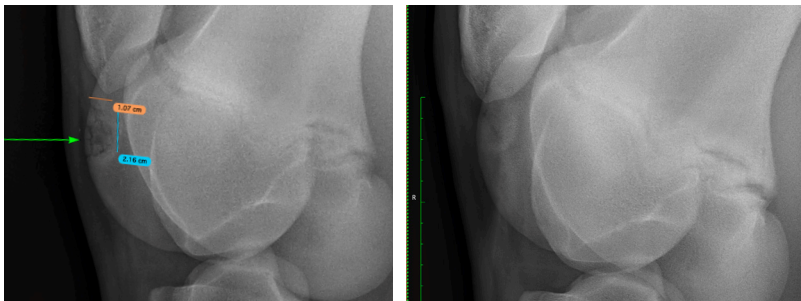
- Emma S

Horse Owner to “Vivi”¹

1. Case Study Disclaimer: Individual clinical example shown for illustrative purposes only. Outcomes may differ between animals and should not be interpreted as typical or guaranteed. This case does not expand or modify FDA-approved labeling. Use according to complete product labeling and the professional judgment of the prescribing veterinarian.

equicenta® CTM in Action

Side-by-side imaging from real-word cases treated for SDF core lesions, tendonitis, and severe tendon injuries.



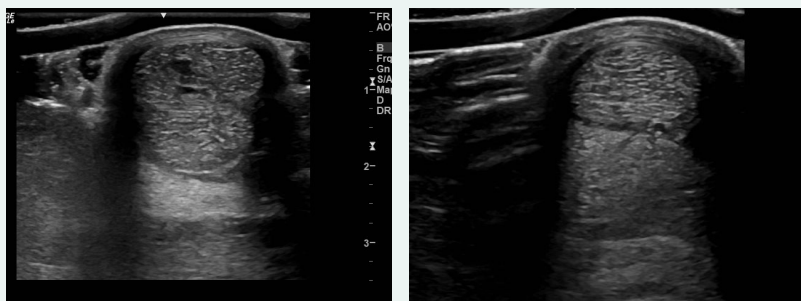
Day 0

Day 60

Species: Equine (horse)
Age: Yearling
Breed: Thoroughbred
Job/Use: Racehorse
Diagnosis: Stifle joint with lateral trochlear ridge osteochondritis dissecans (OCD)
Treatment: Single injection equicenta® CTM at Day 0

"Note filling of defect with smooth surfaces, and surgery was avoided"

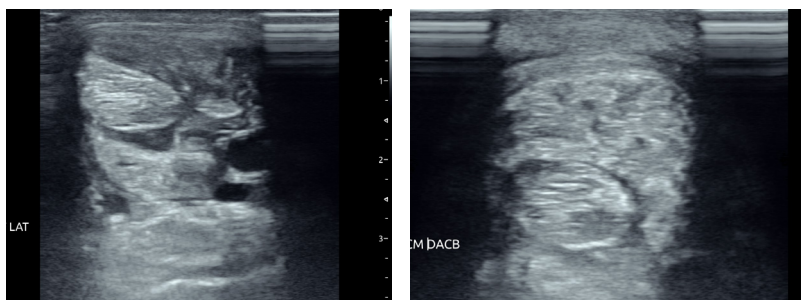
- Treating Veterinarian



Day 0

Day 60

Species: Equine (horse)
Diagnosis: Superficial digital flexor tendon, Tendonitis
Treatment: Single injection equicenta® CTM at Day 0



Day 0

Day 60

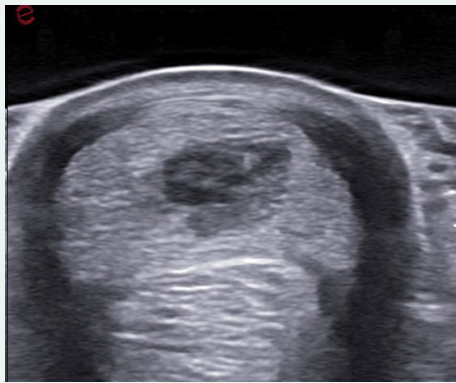
Species: Equine (horse)
Age: 22
Breed: Quarter Horse
Diagnosis: Severe Superficial digital flexor injury
Treatment: Single injection equicenta® CTM at Day 0

Case Study Disclaimer: Individual clinical example shown for illustrative purposes only. Outcomes may differ between animals and should not be interpreted as typical or guaranteed. This case does not expand or modify FDA-approved labeling. Use according to complete product labeling and the professional judgment of the prescribing veterinarian.

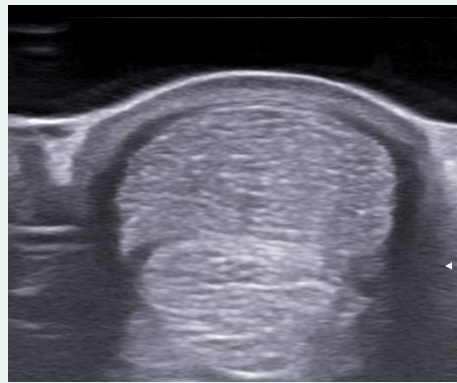


equicenta® CTM in Action

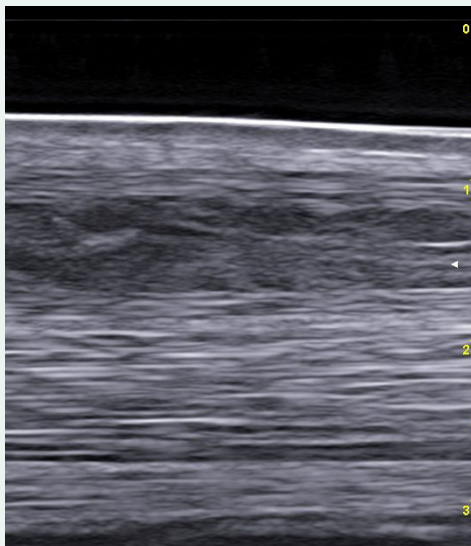
Side-by-side imaging from real-word cases treated for SDF core lesions, tendonitis, and severe tendon injuries.



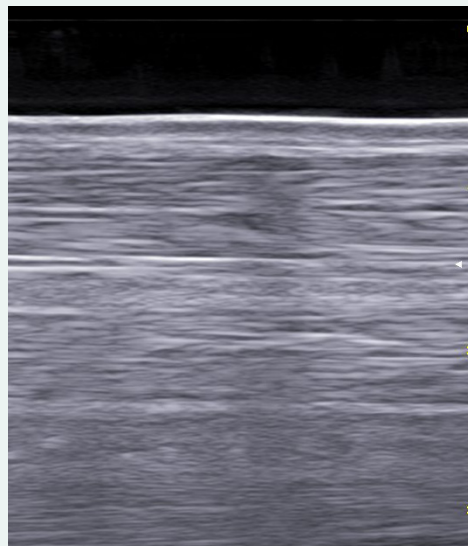
Day 0
Cross-Section
RF SDFT



Day 60
Cross-Section
RF SDFT



Day 0
Longitudinal
RF SDFT



Day 60
Longitudinal
RF SDFT

Species: Equine (horse)
Age: 3
Breed: Thoroughbred
Job/Use: Racehorse
Diagnosis: Superficial digital flexor tendon, core lesion
Treatment: Single injection equicenta® CTM at Day 0

Case Study Disclaimer: Individual clinical example shown for illustrative purposes only. Outcomes may differ between animals and should not be interpreted as typical or guaranteed. This case does not expand or modify FDA-approved labeling. Use according to complete product labeling and the professional judgment of the prescribing veterinarian.



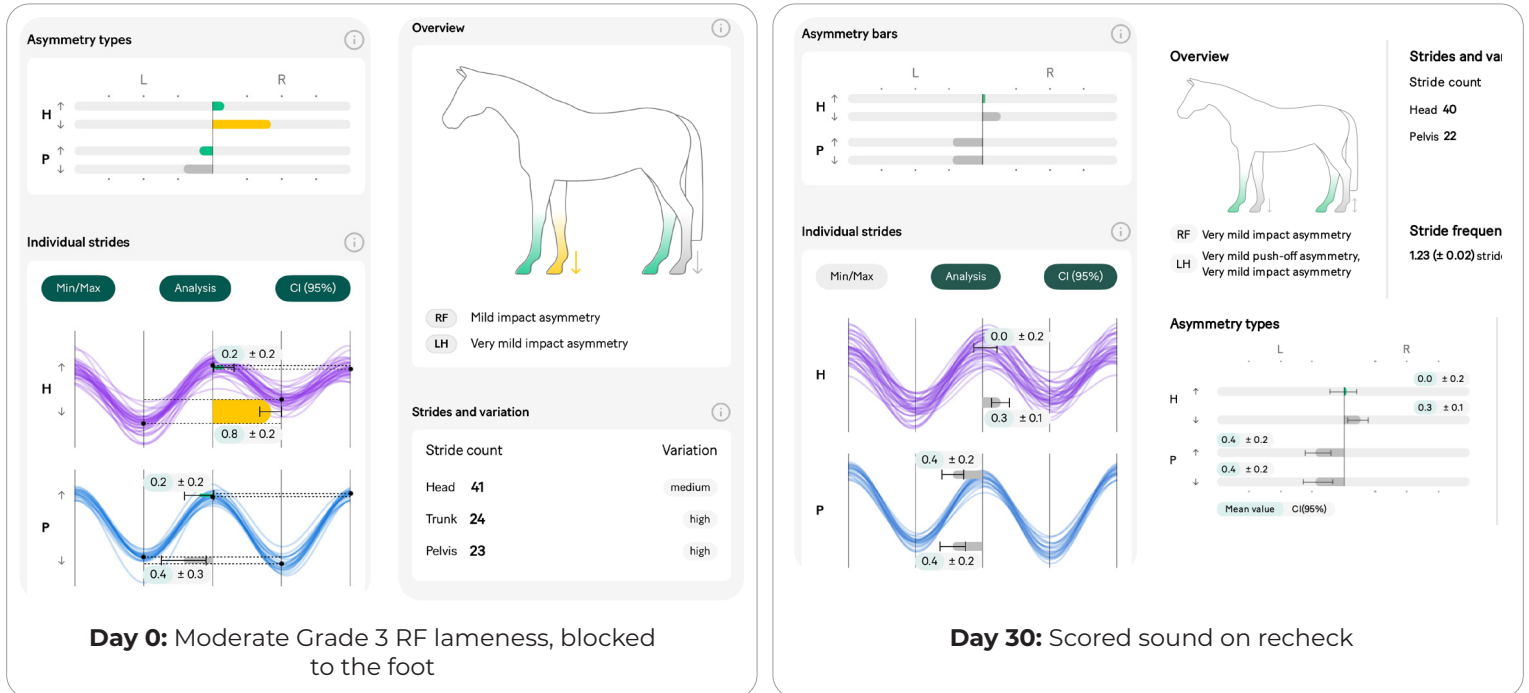
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Grade 3 Right Front Lameness with Coffin Joint Osteoarthritis

Blocked to the foot, radiographic OA, and a history of surgically fused hind hock joints.



Presentation + Day 0 image

The horse presented with a moderate Grade 3 right front lameness that blocked to the foot. Radiographs identified osteoarthritis of the coffin joint. The horse also had a history of left hind DIT and TMT joints surgically fused in a prior procedure. Gait analysis at presentation showed mild impact asymmetry in the right front and very mild impact asymmetry in the left hind.

Treatment Approach

- equicenta® CTM injection at the tear site
- Ongoing monitoring and progressive load

Day 30 — Early Response

At the 4-week recheck, the horse scored sound. Gait analysis showed very mild impact asymmetry in the right front and very mild push-off and impact asymmetry in the left hind — both within a low, stable range. The LH surgically fused DIT/TMT remained unchanged from baseline.

Clinical Takeaway

Joints respond on the average 4 weeks after injection of equicenta® CTM.¹



equicenta® CTM

Want to see more cases?

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Grade 4 LF Lameness with Tendon Injury

DDF tendon tear and fibrosis with a new tear.



Day 0: Grade 4 LF lameness, visible swelling at suspensory branch



Day 30: Reduced swelling, early improvement in symmetry

Presentation + Day 0 image

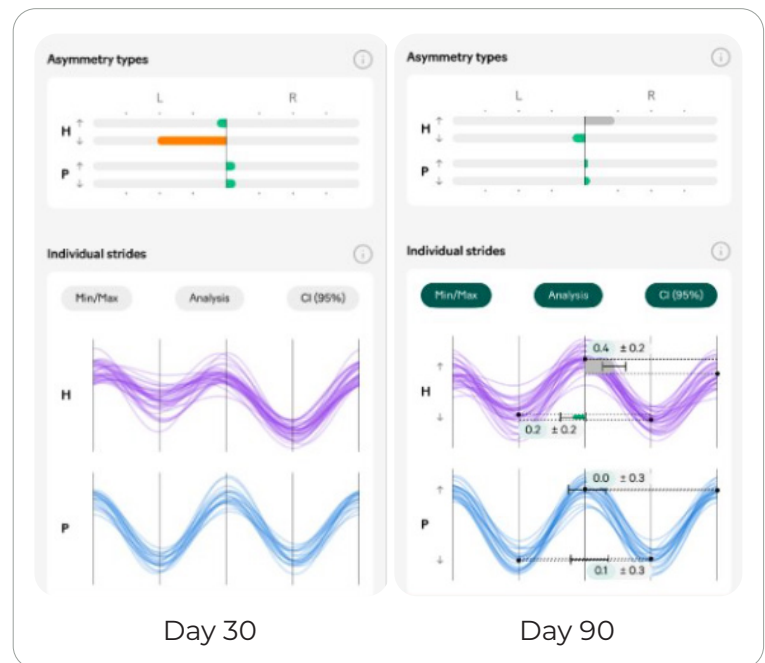
A performance horse presented with Grade 4 LF lameness, a DDF tendon tear. Ultrasound showed fibrosis alongside the new tear, indicating both chronic and acute injury in the same limb.

Treatment Approach

- equicenta® CTM injection at the tear site
- Corrective shoeing to reduce tendon strain
- Stall rest followed by controlled rehabilitation
- Ongoing monitoring and progressive load

Day 30 — Early Response

At early re-evaluation, gait analysis indicated an early positive response to treatment. Symmetry and movement patterns showed measurable improvement. The horse remained within a controlled rehabilitation phase.



Day 90 — Cleared for Work

At follow-up, the horse was cleared for return to under-saddle work. Symmetry was restored and limb function had improved significantly. The horse progressed back toward performance and was preparing for transition to Wellington.

Clinical Takeaway

Complex musculoskeletal tissue injury rarely responds to single-modality intervention. This case demonstrates the value of biologic support within a structured, multimodal recovery plan.¹



Scan for Full Article

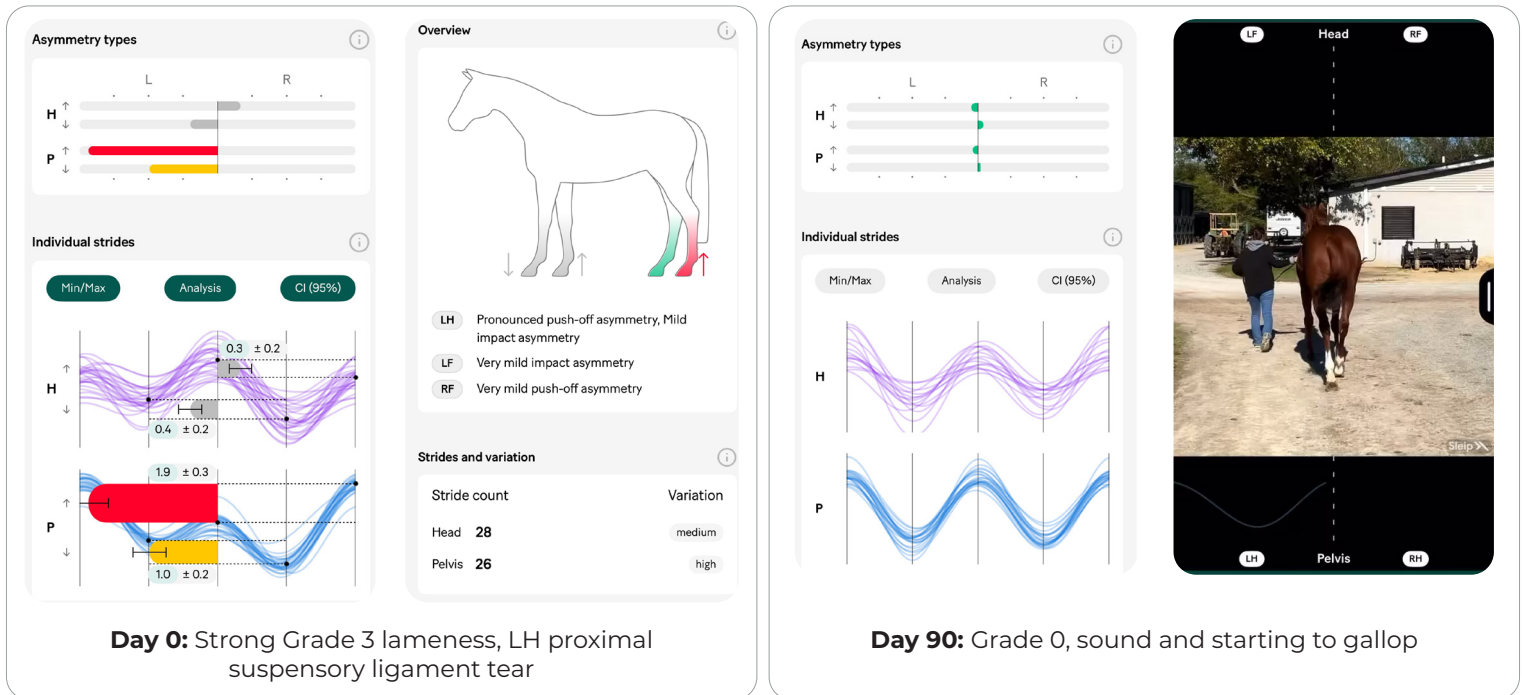
Front. Vet. Sci. 13:1833933 (2026). doi:10.3389/fvets.2026.1833933
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Thoroughbred Racehorse with Left Hind Proximal Suspensory Ligament Tear

Strong Grade 3 lameness after a race, treated with equicenta® CTM.



Presentation + Day 0 image

A Thoroughbred racehorse went lame after a race. Diagnostic work-up identified a proximal suspensory ligament (PSL) tear in the left hind, graded as a strong Grade 3 lameness. Gait analysis at presentation showed pronounced push-off asymmetry with mild impact asymmetry in the left hind, and very mild push-off asymmetry in the left front.

Treatment Approach

- equicenta® CTM injection at the LH proximal suspensory ligament tear

Day 90 — Cleared to Gallop

At the 90-day recheck, the horse was Grade 0 and sound, with gait analysis showing values back near baseline across all measures. The horse had progressed to starting gallop work.

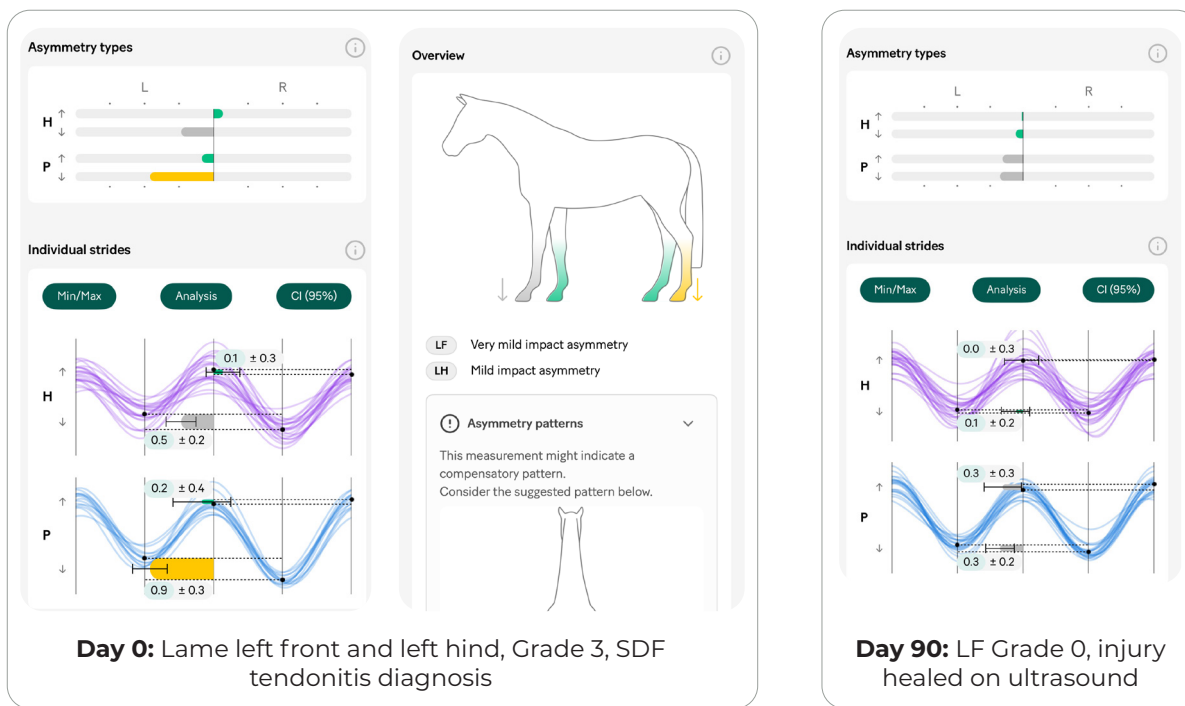
Clinical Takeaway

A proximal suspensory ligament tear in a racing Thoroughbred is a high-stakes injury for return to performance. This case shows a strong Grade 3 lameness returning to Grade 0 and cleared for galloping within 90 days of equicenta® CTM treatment.¹



Bilateral Front Limb Lameness with Superficial Digital Flexor Tendonitis

Grade 3 lameness in both front limbs, treated with equicenta® CTM at the site of injury.



Presentation + Day 0 image

This Standardbred race horse presented after a race lame in both the left front and left hind, graded as a Grade 3 lameness. Diagnostic imaging confirmed acute superficial digital flexor (SDF) tendonitis in the left front. Gait analysis at presentation showed very mild impact asymmetry in the left front and mild impact asymmetry in the left hind attributed to compensatory hock soreness.

Treatment Approach

- equicenta® CTM injected at the site of injury
- Concurrent EPM medical management for an unrelated diagnosis identified during work-up
- Stall rest and progressive return to training

Day 90 — Results

At the 90-day recheck, the left front was Grade 0 and the original injury had healed on ultrasound. Separately, the horse showed mild soreness in both hind limbs, unrelated to the original tendon injury, which was addressed with a triamcinolone injection to the hocks. The horse had started training and was reported ready to race.

Clinical Takeaway

Bilateral SDF tendonitis carries real risk of prolonged downtime, especially in a horse still working toward a racing career. This case shows a clean ultrasound-confirmed healing outcome by Day 90, with the horse back in training and cleared to race.¹



Scan for Full Article

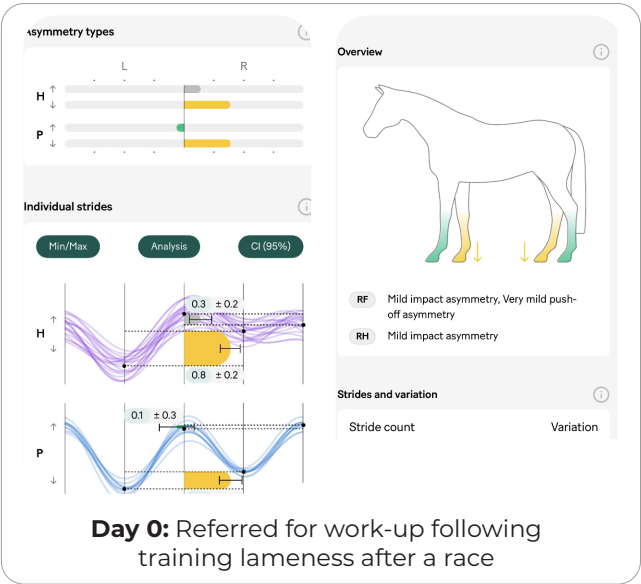
Front. Vet. Sci. 13:1833933 (2026). doi: 10.3389/fvets.2026.1833933
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3-Year-Old Thoroughbred Racehorse — Lameness Work-Up After a Claiming Race

Referred for evaluation after finishing 3rd in a maiden claiming race, with imaging findings pointing to cartilage damage.



Presentation + Day 0 image

A 3-year-old Thoroughbred racehorse, finished 3rd in a maiden claiming race and subsequently showed a lameness issue in training. The horse was referred for a full diagnostic work-up. Findings included a right front block to the metacarpophalangeal joint (MCJ) with no significant findings in the right hind, and radiographs showing C3 sclerosis, with imaging likely indicating cartilage damage. Gait analysis at presentation showed impact asymmetry patterns in the right front and hind.

Treatment Approach

- equicenta[®] CTM injected at the site of injury
- Controlled rehabilitation with progressive return to breezing
- Ongoing monitoring through the return to racing

StatisticsResultsWorkoutsRating HistoryAuction History						
Track	Date	Course	Distance	Time	Note	Rank
Horseshoe Indianapolis	10/18/2025	Dirt	4F	50.00	Breezing	37/62
Horseshoe Indianapolis	10/9/2025	Dirt	4F	48.80	Breezing	8/26

StatisticsResultsWorkoutsRating HistoryAuction History									
Track	Date	Race	Race Type	Breed	Finish	Rating	E	Chart	Video
Horseshoe Indianapolis	9/18/2025	5	Maiden Special Weight	TB	8	N/A	51		
Horseshoe Indianapolis	9/3/2025	5	Maiden Special Weight	TB	6	N/A	63		
Fair Grounds	3/6/2025	7	Maiden Optional Claiming	TB	3	N/A	70		

Day 90/180: Sound, breezing, and back to racing

Day 90 and Day 180 — Return to Racing

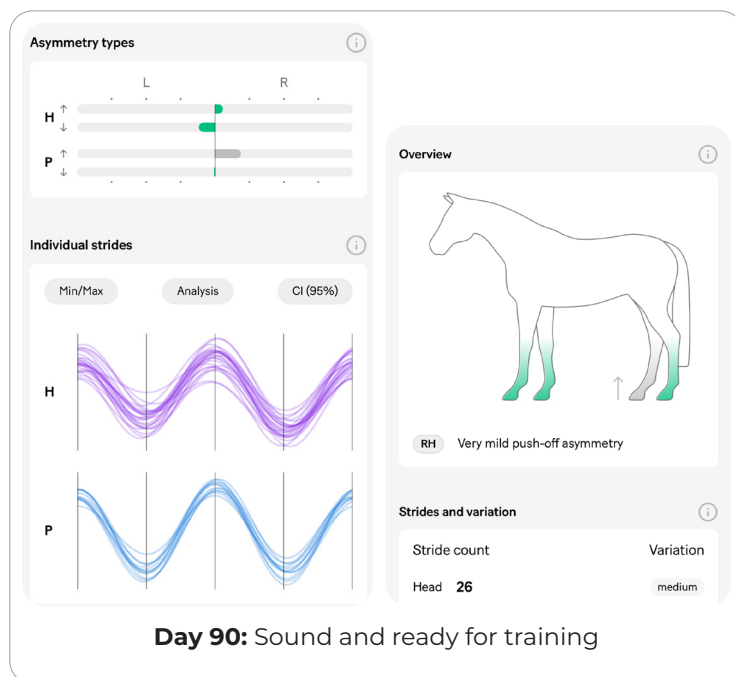
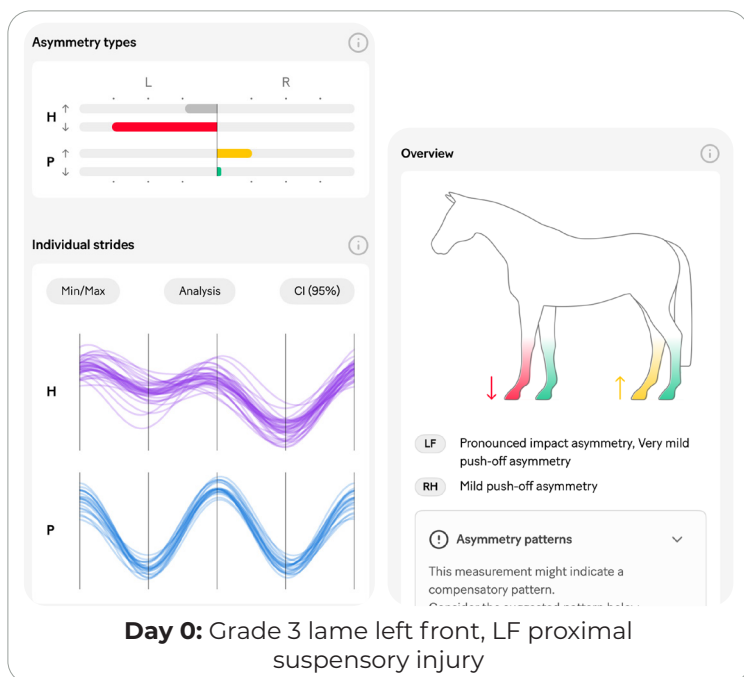
At the Day 90 evaluation, the horse was sound and ready for breezing. By Day 180, the horse was back to racing, with sustained performance confirmed via Equibase.com — public race records show three subsequent starts (finishing 8th, 6th, and 3rd) along with recorded breeze workouts.

Clinical Takeaway

This case tracks a return-to-racing timeline using public performance records as an objective outcome measure, rather than only gait-analysis data — a useful complement to the clinical metrics used elsewhere in this booklet.¹

2-Year-Old Thoroughbred in Training — Grade 3 Lane Left Front

Proximal suspensory injury in a young racehorse, treated with equicenta® CTM.



Presentation + Day 0 image

2-year-old Thoroughbred racehorse in training, presented with a Grade 3 lameness in the left front due to a proximal suspensory injury. Gait analysis at presentation showed pronounced impact asymmetry with very mild push-off asymmetry in the right front, and mild push-off asymmetry in the right hind.

Treatment Approach

- equicenta® CTM injected at the site of injury
- Structured rehabilitation, including swimming
- Progressive return to training with ongoing monitoring

Day 90 — Sound and Ready for Training

At the 90-day recheck following equicenta® CTM injection and rehabilitation, the horse was sound and ready to return to training. Gait analysis showed asymmetry values had dropped to very mild, a marked improvement from the pronounced asymmetry at presentation.

Clinical Takeaway

A young racehorse with a proximal suspensory injury returning from pronounced asymmetry to sound, training-ready status within 90 days demonstrates the value of early, targeted biologic intervention in developing athletes.¹

Statistics	Results	Workouts	Auction History
------------	---------	----------	-----------------

Track	Date	Race	Race Type	Breed	Finish	E
Fair Grounds	11/27/2025	7	Maiden Special Weight	TB	6	30

Statistics	Results	Workouts	Auction History
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Track	Date	Course	Distance	Time	Note	Rank
Fair Grounds	11/23/2025	Dirt	3F	38.40	Breezing	15/18
Fair Grounds	11/16/2025	Dirt	4F	49.00	Breezing	10/57
Fair Grounds	11/09/2025	Dirt	5F	1:01.20	Breezing	3/24
Fair Grounds	10/30/2025	Dirt	4F	48.40	Breezing	2/29
Fair Grounds	10/23/2025	Dirt	4F	49.60	Breezing	2/7
Fair Grounds	10/17/2025	Dirt	4F	51.00	Breezing	21/28
Keeneland	05/31/2025	Dirt	4F	47.40	Breezing	2/57
Keeneland	05/23/2025	Dirt	4F	48.60	Breezing	11/45
Keeneland	05/16/2025	Dirt	3F	35.20	Breezing	1/9
Keeneland	05/08/2025	Dirt	3F	38.00	Breezing	13/36



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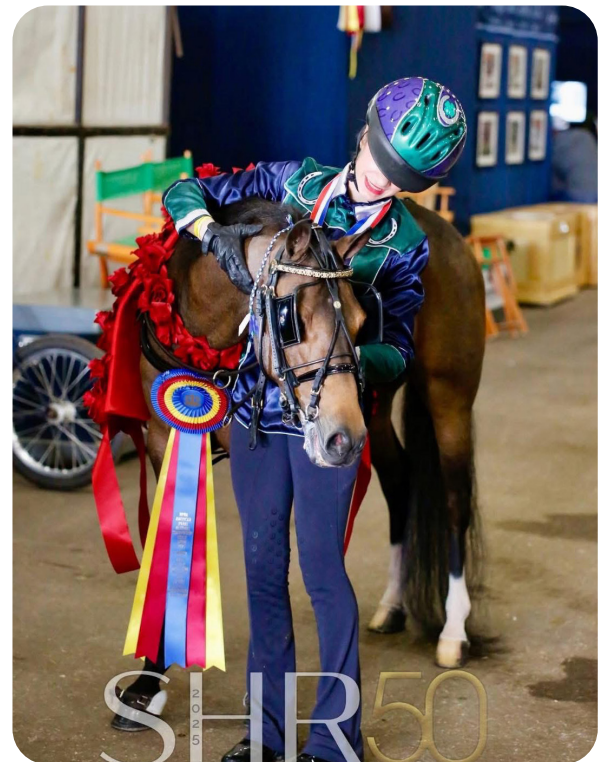
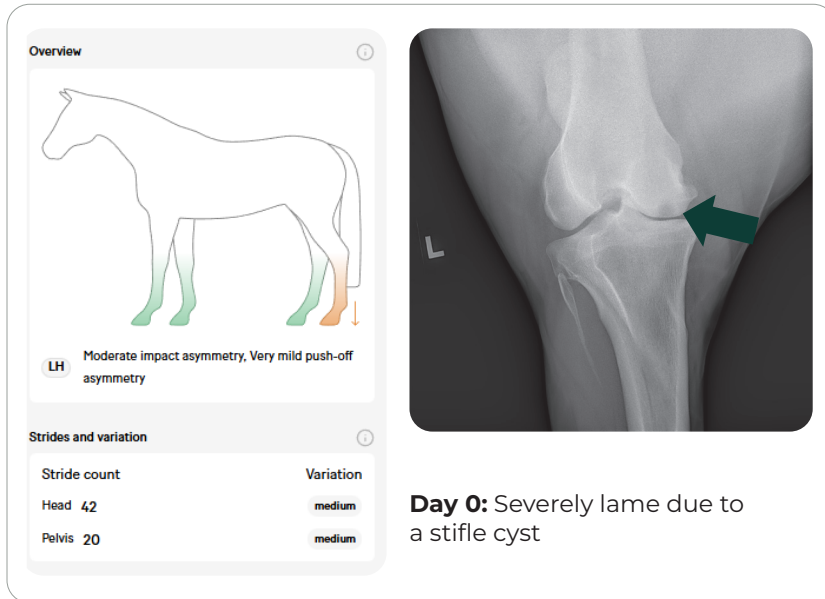
Front. Vet. Sci. 13:1833933 (2026). doi:10.3389/fvets.2026.1833933
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Hackney Pony — From Severely Lame to Champion in 3 Months

A stifle cyst sidelined a show pony; equicenta® CTM supported a return to the winner's circle.



Day 90: Captured the roses in the AHHS Youth Medallion Road Pony 13 and Under National Championship

Presentation + Day 0 image

A Hackney show pony presented severely lame due to a stifle cyst.

Treatment Plan

- equicenta® CTM injected at day 0

3 Months — Champion

Within three months, the pony returned to the show ring as champion.

Clinical Takeaway

A stifle cyst is a common but potentially career-limiting diagnosis in performance horses. This case shows a pony going from severely lame to champion-level performance within three months of equicenta® CTM treatment.¹



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The *All-In-One* Solution

Innovation, **Backed by Science & Evidence**



The Power of **Nature's Own Recipe**, Engineered in One Single Injection at Your Fingertips





The *All-In-One* Solution

Innovation, **Backed by Science & Evidence**

Published in BMC Medicine & Frontiers: Veterinary Science | ISO 13485 Certified Quality System | Patent Pending

Outperformed Standard of Care³

3X more likely to be pain-free lameness cumulative improvement — better on every clinical outcome by Wk 12 ($p < 0.001$).³

Safe by Every Measure³

4.8X fewer injection reactions than standard of care. Zero vet-reported serious adverse events attributed to equicenta® CTM³

2 Peer-Reviewed Studies

Safety & consistency (BMC Medicine).
Clinical efficacy (Frontiers: Veterinary Science).



Return To Performance

2.72:1 odds of returning to prior competition ($p < 0.006$) — the strongest effect in the study (Cohen's $d = 2.23$).³

Real Tissue Repair

36% of ligaments remodeled on ultrasound vs 0% for SOC — the only treatment achieving clinical and structural recovery.³

Trusted in the Field

600+ doses administered across 65+ practices, backed by published evidence.

138 HORSES · 195 CONDITIONS · 8 PRACTICES · 26 WEEKS

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Karin Williamson

Director of Business Development

Central + Western Region

karin@equineperformancelabs.com

(559) 709-5962

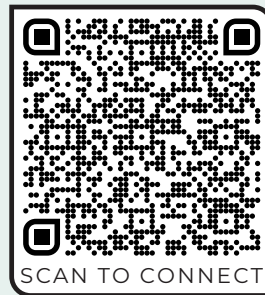
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Important information: equicenta® CTM is not for human use. Not for use in food-producing animals. The results shown are from a single prospective controlled clinical study in performance horses; individual outcomes may vary. Source: Bertone AL, et al. Front Vet Sci. 2026. doi: 10.3389/fvets.2026.1833933 · AVMA VCT #26005934 · IACUC ETCR-2024-0329. 1. Case Study Disclaimer: Individual clinical example shown for illustrative purposes only. Outcomes may differ between animals and should not be interpreted as typical or guaranteed. This case does not expand or modify FDA-approved labeling. Use according to complete product labeling and the professional judgment of the prescribing veterinarian. 2. Bertone AL, et al. Cryopreserved equine umbilical cord tissue allograft characterization and biocompatibility in vivo in musculoskeletal tissues: a controlled study. BMC Med. 2025;23:439. doi:10.1186/s12916-025-04231-7. PMID: 40702469. 3. Bertone AL, et al. Cryopreserved umbilical allograft (equicenta® CTM) improves clinical outcomes compared with clinician-selected standard of care in performance horses: a pragmatic prospective controlled trial. Front Vet Sci. 2026;13:1833933. doi:10.3389/fvets.2026.1833933. 4. EVIDENTIARY NOTE Tissue-remodeling scoring (TRS), Slep and combined-limb kinematics, and all condition, product-class, APS, hemoderivative, and rescue contrasts are or post hoc analyses and, in the manuscript's words, mainly hypothesis-generating. The pre-specified primary evidence is the controlled clinical outcomes, performance odds, and safety. equicenta® CTM is investigational and not FDA CVM approved.